PRE-RELEASE

The Wrong Clock

Why the Medical plan should follow the evidence, not the corporate calendar.

Christian Wasmer & Friederike Winter · Consilience · October 29, 2026

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1 · The downside of following corporate calendars

An asset's Medical plan is typically built and updated along the annual corporate calendar. Timelines mainly fit the budget cycle, and the "final" plan needs to be fully signed off before a new (fiscal) year begins. As a result, the plan's elements will be fixed without much consideration for when the underlying evidence may change.

For example, an asset's 2027 Medical plan developed in H1 of 2026 had to be ready for leadership sign-off and budget submission by ~September. As the field's main congress is still to happen in Q4, the plan may be outdated by the time it is put into action, leaving executing functions uncertain about its relevance.

While relevant for strategic planning in general, this is particularly problematic for Medical Affairs, as one of its key purposes is to interpret scientific evidence and discuss it with the medical community. So it is critical to consider the latest trial readouts, congress presentations, guideline updates, and observations from interactions with thought leaders, the wider HCP community, and patients. And while landscape and congress monitoring are well established, updating an existing plan off-cycle is often seen as too much effort.

In this white paper we argue that letting science, not the corporate calendar, set the schedule for Medical planning not only improves the plan's relevance, but is also feasible in today's corporate environment.

2 · Focusing the adaptations where they are needed

Most Medical plans follow a similar hierarchy of elements, with a vision or aspirational narrative "on top," strategy and tactics at its core, and materials supporting the execution. Some elements tend to be relatively durable, e.g., the asset's aspirational narrative and the medical community it aims to address. Strategy may need adjustment more often, while most signals only affect tactics and materials (see Box A, below).

While changes should be considered part of a living Medical plan, it is critical to focus updates on the elements that really need to change for the plan to remain effective. A reduced annual cycle is retained for a review of the year, as well as aspects that are less tied to changes in the environment, such as budget reconciliation, corporate administrative requirements, and individual objectives.

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Box A · Levels of a Medical plan

An asset's Medical plan is typically structured into 4 hierarchical levels: aspirational narrative, strategy, tactics, materials.

Level	What it defines	Changes	Decided by
Aspirational narrative	an asset's aim: whom to benefit and how	rarely	Medical leadership in discussion with cross-functional leadership
Strategy	the path, and what is required to follow it	sometimes	full Medical team; input from cross-functional leadership if relevant
Tactics incl. interdependencies	anything needed to stay on the path	often	all subfunctions involved and dependent
Materials	any content (logic, evidence, references, etc.) used in tactics	continually	typically one subfunction

A workable aspirational narrative

is a target product profile spelled out in medical and scientific form: concrete enough to yield testable hypotheses and exclusively supported by positions that data can confirm or falsify. (Otherwise, it is an intention, not a scientific position, and should have no place in Medical Affairs.)

3 · A recipe for evidence-driven Medical planning

Integrating insights into a Medical plan follows a 3-step process — assessment, decision, implementation. It begins once a signal has been detected (Figure 1).

Signal detection is achieved through existing processes: for discrete events such as congresses, competitor readouts, and guideline updates, a (Medical) intelligence function disseminates updates; for signals such as field insights or Medical Information requests, the relevant teams provide updates at a set cadence, typically monthly to quarterly.

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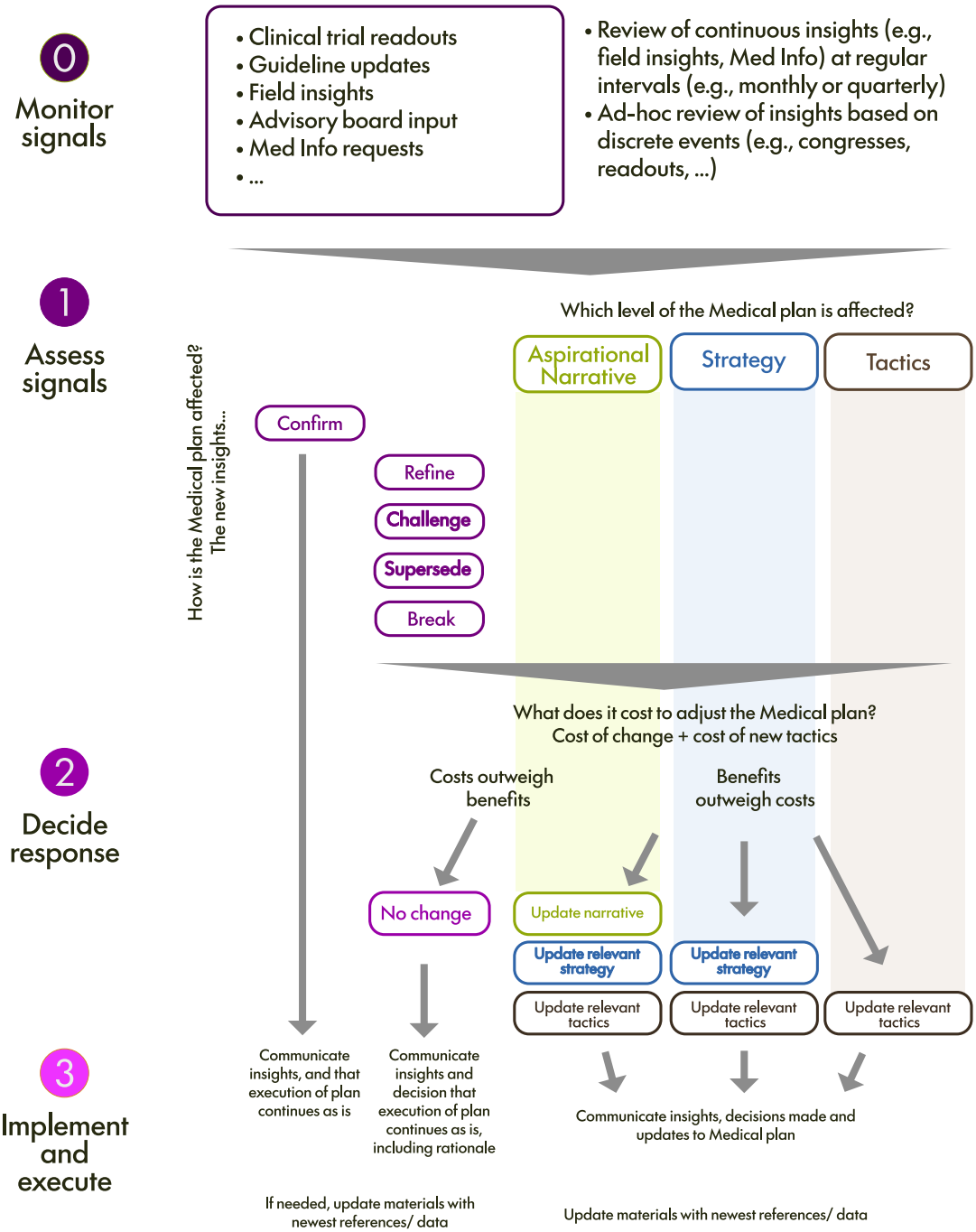


Figure 1 · From signal to execution. *The 4 stages by which an incoming insight reaches the Medical plan, from continuous monitoring through to execution.*

How to read this figure: *Monitoring (0) runs continuously through channels already in place — trial readouts, guideline updates, field insights, advisory board input, Medical Information requests — reviewed either at a set cadence or ad-hoc around discrete events. Assessment (1) answers two questions at once: how the insight relates to the plan (confirm, refine, challenge, supersede, break) and which level it reaches (aspirational narrative, strategy, tactics). A signal that only confirms the current position exits directly to implementation. Everything else passes to the decision (2), which weighs the cost of change plus the cost of new tactics against the benefit. Where costs outweigh benefits, the plan is left as is; where they do not, the revision cascades downward: a change to the narrative pulls strategy and tactics with it, a change to strategy pulls tactics. Implementation (3) always includes communication, whether or not the plan itself changed.*

Note: the diagram labels the monitoring stage 0 because it precedes the 3-step process described in the text rather than forming part of it.

The first step then is assessment: which level the signal affects, and how. A signal may confirm the current position, refine it, challenge it, supersede it, or break it, and it does so at the level of materials, tactics, strategy, or, rarely, the aspirational narrative.

Step 2 is the decision for or against updating the plan, made by weighing the cost of change against the value of the potential update. Cost here means organizational effort as well as money. A running trial, a committed publication plan, or an approved field strategy can never be altered "for free." Cross-functional dependencies may increase the cost of change.

It is important to separate two aspects often discussed as one. "No change is needed" is an assessment (new information does not change our position). "Change is not worth making" is a decision (new information affects the position, but cost exceeds the potential value). It is never worth (re-)debating the evidence if the real disagreement is about cost.

Revisions to the aspirational narrative are rare because a narrative well chosen is durable and because change at that level is expensive. It may be broken outright if, e.g., a pivotal trial fails. More frequently an assumption of the narrative is superseded, e.g., a new standard of care emerges. Of note, a narrative the evidence has superseded may still be held for a time, but that needs to be an explicit decision on the record and communicated to the full cross-functional team as such. Any change to the aspirational narrative will likely also affect other functions' plans, so these changes may be proposed by Medical Affairs, but should be aligned with relevant cross-functional leadership.

Finally, in step 3, changes are written into the plan, downstream materials are updated, and execution continues. Clear communication of such updates is critical to maintain the team's buy-in and alignment.

Table 1 · Example signals, assessments, and decisions

Signal	Arrival	Assessment: level to change	Assessment: direction	Decision
Confirmatory real-world result, consistent with the position	discrete	materials	confirms	update materials; no re-plan
Educational event feedback is positive and indicates intent to change patient care	discrete	tactics	refines	roll out the educational event to additional stakeholders
Competitor Phase 3 reads out positive in an adjacent indication	discrete	strategy	challenges	reprioritize emphasis across evidence generation, field, and communication; may be budget-neutral
Field observation of an emerging use pattern	continuous (reviewed on cadence)	strategy	refines	investigate; revise now or hold for the next periodic review
Guideline body signals a change in standard of care	discrete	aspirational narrative	supersedes	Medical leadership proposes whether and when to revise, discusses with cross-functional leadership, communicates to the team

Illustrative. Actual reading depends on asset and context.

4 · Fitting it into current corporate environments

Prerequisites

Which prerequisites matter for implementation success will depend on the corporate context; below are the 4 we consider most relevant.

In some organizations, and for assets nearing commercial launch, the Medical plan will feed into an asset's brand plan. **Brand plan integration** is often limited to including the full Medical tactics in the context of the annual brand planning exercise. Updating the Medical plan more than once a year may create some degree of misalignment, which may be acceptable for relatively minor, tactical updates. More substantial changes at the strategy or aspirational narrative level, on the other hand, are very likely to render the brand plan outdated as well. While the signal may often originate from Medical Affairs, in these cases, the cross-functional asset leadership will need to decide on updating the full brand plan and asset strategy. Discussing evidence-driven planning and example scenarios with cross-functional leadership upfront will be beneficial.

The proposed approach to Medical planning has two components: (1) monitoring for relevant signals, which is almost always useful, and (2) responding by revising the plan, which requires thoughtful consideration. Setting the **right threshold for when a signal actually leads to a response** is critical and needs to be fine-tuned individually. Frequent revisions can indicate a volatile environment or be due to miscalibration. Setting the threshold too high can lead to signals accumulating into an unexpectedly large shift, mitigated by keeping track of unaddressed signals for the next scheduled or an additional ad-hoc review. Overall, the "right" update frequency and hence the "right" threshold will depend mainly on therapeutic area, target indication, and asset context. Medical plans for assets in development, peri-approval, or early in a competitive market may thrive on frequent updates (in the range of 4 times per year). For mature products in non-competitive areas, generics, and biosimilars, accumulating updates for an annual revision may be reasonable.

Another consideration is the **level of budget flexibility** at a given company. On the more rigid side, strategic revisions may only be allowed to change budget allocation within a given budget line. Tactics may be refined, in most cases also reprioritized, but not expanded (or reduced) in effort. Reallocation typically sits within a budget holder's authority. Adding to (or removing from) the total budget above a certain limit will always require leadership sign-off and should remain limited to the most impactful events. It is worth confirming the limits and scope of budget holder authority upfront.

Existing **legal processes** will need to be integrated and adjusted, which may also present an opportunity. Legal review of the Medical plan now applies per revision rather than annually. As revisions tend to be smaller and more clearly motivated, legal and compliance teams may remain more familiar with the plan, and less time and fewer iterations should be needed per update. On the flip side, the total documented material subject to disclosure or discovery may increase. The net effect will vary by company.

Barriers

While we believe evidence-driven Medical planning is superior in many scenarios, the approach will require a flexibility that many organizations have lost as they have grown: the ability to adjust course with limited bureaucracy so revisions can be identified, decided, and funded at the speed the evidence demands. Without it, a trigger-based plan reverts to an annual one within a cycle. Strong leadership support, clear decision rights, piloting, and communicating early results will be key to successful implementation.

5 · Summary

A Medical plan updated as new evidence and insights arrive will become a reliable reference for the Medical Affairs team, available at all times. The annual cycle continues to do what it was designed for: reconcile the budget, review the year, and set individual objectives. Medical Affairs holds its external communication to the standard of the latest evidence; we believe its planning should meet the same standard.

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A note on how we work with AI

This publication was developed with AI support. The authors defined the framework, developed the figure, and drafted and edited the publication. Claude Opus 5/5.5 supported secondary research, drafted sections, conducted consistency checks and proofreads at the authors' direction, and formatted the final output according to Consilience's brand guidelines. Consilience team members reviewed and approved all content. AI helped improve quality and speed.

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